

THE PRESENT MOMENT IN PSYCHOTHERAPY AND EVERYDAY LIFE.
By *Daniel N. Stern*. New York: W. W. Norton, 2004, xxi +
283 pp., \$29.95.

The relational nature of mind and therapeutic process is suggested by the increasingly generative collaboration between developmental researchers and clinicians. Close observation of infant-parent interaction provides new hard data on which to base hypotheses about therapeutic process. Relational ideas, especially from these sources, have often been less immediately obvious to clinicians. Sitting *in* the relationship, the clinician finds it hard to experience the *relationship* as much as he or she experiences the *patient*. Intrapsychic models tend to make sense from the analyst's vantage point. With the model of early infant-parent interactions, relational patterns become more intuitively obvious, understandable, and compelling.

This is what Daniel Stern and his colleagues (Sander, Nahum, Harrison, Lyons-Ruth, Morgan, Bruschweiler-Stern, and Tronick)—as well as others in the relational/developmental movement like Lichtenberg, Seligman, and Beebe and Lachmann—have been trying to help us with, conceptually, over the past two decades. In his latest book, *The Present Moment in Psychotherapy and Everyday Life*, Stern attempts something akin to Freud in the *Project*—a model of intersubjective consciousness and interactive behavior, based on biological research findings. In effect, Stern is going back to fill in scientifically and conceptually around ideas presented in his contributions with the Boston Change Process Study Group, beginning with the widely read “Non-interpretive Mechanisms in Psychoanalytic Therapy: the ‘Something More’ than Interpretation” (Stern et al. 1998). Many of us read this paper, saying, “Of course, we always knew there was another channel of communication. Glad to have someone point it out.” But Stern was saying something more radical, that the second channel is at least as important as the verbal, interpretive one.

In the 1998 paper Stern proposed that alongside the familiar processes of free association, resistance, transference, and interpretation there exists another set of experiences and interactions contained in nonverbal and paralinguistic realms. Patient and analyst move along, affecting each other in complex ways, cuing each other, and coming together more intensely for brief periods he calls “now moments.” Even more intense engagements—ones that permanently alter both parties as

well as the relationship—he calls “moments of meeting.” The paper also proposed how the dyadic process functions as a dynamic system, constantly fluctuating between equilibrium and deliberate destabilizations from one side or the other.

The Present Moment is an expansion of the original papers from Stern’s group, fleshing out his study of how humans experience themselves in time—in brief chunks, it turns out, lasting three to ten seconds. He relates the phenomenology of this constantly shifting present to its neurobiological basis, and then to dyadic experience in psychotherapy. For analysts this is where things get really interesting; there have been relatively few careful descriptions of process at the level of analysis that Stern discusses, especially attempts to conceptualize the interactive process as such. For Stern, “Uh”s and “I don’t feel like talking”s are not so much resistance manifestations, as statements about a person’s momentary subjective position in the dyad and vis-à-vis their subject matter. Such statements are part of a dynamic system, cuing the analyst with their ambiguity, inviting the possibility of joining in silence or encouraging with a “Hmm?” leading to a further step that may “move along” the process. In this way the dyad may or may not gradually move toward greater intensity—now moments—or even those highly significant change points—moments of meeting.

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What is radical about Stern is the idea that a significant aspect of therapeutic activity takes place in the realm of what he calls “implicit knowing.” As distinguished from the long-standing psychoanalytic assumption that the crucial issues in mutative process are to be “uncovered”—topographically brought from the dynamic unconscious into conscious awareness, and interpreted verbally—Stern’s implicit knowledge is out of awareness because it is fleeting and often in a non-verbal realm. It can, however, be accessed through careful, retrospective questioning and/or self-reflection. Most important is Stern’s argument that change occurs in the interaction of two parties in this realm of implicit knowing, particularly at “moments of meeting.” This is a recasting of the theoretical structure of analysis, a recasting that puts the emphasis on what is hard to see because it is momentary and rarely in the secondary process realm, but not because it is concealed behind a barrier of repression.

In *The Present Moment* Stern goes beyond previous work to describe and illustrate the detail of momentary experience, individual

and dyadic. He builds extensively on current research in neurobiology and cognitive psychology, although it must be pointed out that a great deal of his conceptualizing is creative and speculative. The first third of the book is an explication of the idea of the present moment, drawing to our attention that we live experience in chunks of a few seconds at a time. Consciousness, like musical phrases and most of our experience, is organized in these chunks, apparently because that is how our brains are built. Each chunk, or moment of experience, is organized as it happens according to patterns we carry around in our brains called “vitality affects.” We have met these before in Stern’s earlier work on “affect attunement” as giving shape to the sorts of expressive gestures that are exchanged by child and mother: “*surging, fading away, fleeting, explosive, tentative, effortful, accelerating, decelerating, climaxing, bursting, drawn out, reaching, hesitating, leaning forward, leaning backward, and so on*” (p. 64). There are echoes here of Freud’s libidinal states, with their qualities that may be found throughout character, and of Erikson’s stages; Stern’s gerundive list is particularly detailed and interactional.

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In the second part of the volume Stern sets the context for the place of implicit knowing in psychic function. For example, in a chapter on motivation he argues that there are two major forces that push for intersubjective engagement. One is the desire for orientation—knowing where we stand in a group, whether dyad, family, or larger entity (hints of attachment theory, object relations, and British Middle School here). The other is to make contact with ourselves, to establish a sense of internal cohesion, definition, and identity. (Here we can recognize the link with Kohut.) These motives establish the energy that moves process along. “As [the therapy dyad] move, they pass through an emotional narrative landscape with its hills and valleys of vitality affects, along its river of intentionality (which runs throughout), and over its peaks of dramatic crisis. It is a voyage taken as the present unfolds. A passing subjective landscape is created and makes up a world in a grain of sand” (p. 172).

This last is an important idea for Stern, drawn from the concept of fractals in chaos theory—that large-scale processes like character and structural change can be witnessed in momentary processes like the ones he is studying. He illustrates this in close examinations of the “present moment” using detailed interviews. People show the same tendencies second by second that they show over the large span of their

lives. The final element Stern needs in order to approach the clinical situation and the issue of therapeutic change is that of the past. He broadens the present moment to include the past, not as a veridical version, but as recorded in the hundreds or thousands of fragmentary memory traces of prior present moments, associatively linked to the present. In the clinical situation this multitude of associations allows room for what Stern calls “sloppiness,” an interactive ambiguity that permits the self and the other a variety of responsive possibilities. These may take the situation forward toward greater intensity of engagement (now moments), or may lead to plateaus or distancing. He illustrates these nicely with brief clinical moments that are convincing.

Old (presumably maladaptive) experiences carried in memory may be gradually replaced by new experiences. They need to be in the same temporal framework as the original, Stern argues, without making exactly clear whether he means that there must be a replication of the rhythm of the original experience, or some other parallel. But it is clear that the change process that he envisions contains a significant element of activity at the level of implicit knowing and intersubjectivity. “With an emphasis on implicit experience rather than explicit content, therapeutic aims shift more to the deepening and enriching of experience and less to the understanding [of] its meaning” (p. 222).

Does Stern’s theory truly conflict with established psychoanalytic theory? Yes and no. Yes, because it forces us to acknowledge whole new areas of activity, motivation, and observation, and introduces new language with which to discuss these. No, because I believe that most of our current psychoanalytic understanding can coexist with Stern’s observations. The two are at different levels of magnification. Metapsychology and traditional clinical theory do not attempt to describe or predict specific moments of change; Stern is attempting to do just that, and in doing so is filling in an important space in psychoanalytic theorizing. Skilled clinicians and clinical teachers have often worked with an awareness of the phenomena that Stern describes. The nonverbal and prosodic aspects of communication have been appreciated and recognized for years; what Stern is beginning to develop is a theoretical structure for legitimizing and talking about this realm. Here are the greatest strengths as well as some of the limitations of *The Present Moment*: strengths, because we begin to have terms for describing way stations in implicit process; limitations, because many of Stern’s terms are improvised to cover gaps in explanatory problems. It is not always

clear which concepts truly have strong backing from the neurosciences, and which are more speculative, introduced out of explanatory necessity.

If we take Stern's book as I think it should be taken—as a report on important work in progress—it is a milestone in psychoanalytic thinking. We now have the beginnings of a way to conceptualize and talk with one another about the “something more than interpretation” that goes on in the process of change. Further, we have a series of models of interaction processes in psychoanalysis—equilibrium maintenance and destabilization; crisis and resolution; “fit” and clash—that create powerful change experiences parallel to, or in relation to, interpretive processes. Many of the connections between existing clinical models of change and Stern's new ideas are not explored in this volume. The possibilities for further clinical and research application are immense, and the psychoanalytic enterprise is vastly enriched.

REFERENCE

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